NORTHGATE MULTISPECIALITY HOSPITAL

22/B, Placeholder Marg, Sample Vihar, New Delhi - 110000 | Ph: 011-0000 0000 | NABH Accredited (sample)

SYNTHETIC SAMPLE - fictional patient, hospital and clinicians. Not a medical record. For software testing only.

DIABETES CLINICAL REVIEW — METABOLIC DECOMPENSATION WARNING

Patient Name: Mr. ARJUN MEHTA	**UHID / MRN:** NG-0098731	**Evaluation Date:** 25/04/2027
Age / Sex: 58 Years / Male	**Prior Review Date:** 19/01/2027	**Acuity Status: ATTENTION REQUIRED**
Consultant: Dr. Neha Example, MD, DM	**Clinic:** Urgent Endocrinology Review OPD	**Vitals: BP 156/96 mmHg, BMI 28.9 kg/m²**

CLINICAL PRESENTATION & NON-COMPLIANCE HISTORY

Patient presents with 4-week recurrence of exertional fatigue, bilateral lower limb oedema, nocturia (3-4 times/night), and blurred vision. Advises cessation of insulin injections 6 weeks ago due to needle anxiety; admits irregular intake of oral metformin and antihypertensive medication. Significant dietary lapse reported. Weight has rebounded to 81 kg (+6 kg gain, BMI 28.9 kg/m²). Blood pressure elevated at 156/96 mmHg in Stage 2 hypertensive range.

LABORATORY MEASUREMENTS — TRAJECTORY WORSENING (COMPARED TO PRIOR VISIT)

Biomarker / Investigation	Jan 2027 (Prior)	Apr 2027 (Current)	Trajectory / Delta	Target Reference
Fasting Plasma Glucose	96.0 mg/dL	**218.0 mg/dL**	**Worsened (+122.0 mg/dL)**	70.0 - 99.0 mg/dL
Glycated Hemoglobin (HbA1c)	6.6 %	**10.4 %**	**Severe Upward Drift (+3.8 %)**	< 5.7 %
Blood Pressure (Systolic / Diastolic)	120/78 mmHg	**156/96 mmHg**	**Uncontrolled (+36/+18 mmHg)**	< 120 / < 80 mmHg
Total Cholesterol	184.0 mg/dL	**265.0 mg/dL**	**Elevated (+81.0 mg/dL)**	< 200.0 mg/dL
LDL Cholesterol (Atherogenic)	96.0 mg/dL	**174.0 mg/dL**	**Atherogenic Surge (+78.0 mg/dL)**	< 100.0 mg/dL
Serum Creatinine (Preserved)	0.9 mg/dL	**1.6 mg/dL**	**Renal Impairment (+0.7 mg/dL)**	0.7 - 1.3 mg/dL
Calculated BMI	26.2 kg/m²	**28.9 kg/m²**	**Weight Rebound (+2.7 kg/m²)**	18.5 - 24.9 kg/m²

URGENT CLINICAL INTERVENTION & ACTION REQUIRED

1. **Acute Glycaemic & Renal Deterioration:** Markedly elevated HbA1c 10.4% and creatinine rise to 1.6 mg/dL indicate rapid loss of cardiometabolic control.
2. **Immediate Pharmacotherapy Re-initiation:** Resume subcutaneous basal-bolus insulin immediately. Metformin dose withheld pending eGFR repeat due to creatinine elevation.
3. **Cardiovascular & Nephroprotection:** Escalate Telmisartan to 80mg daily; add Amlodipine 5mg for Stage 2 hypertension. Resume high-intensity Atorvastatin 40mg.
4. **Nephrology & Retinal Referral:** Emergent spot urine ACR and renal ultrasonography scheduled within 72 hours.

Dr. Neha Example, MD, DM (Endocrinology)

Senior Consultant Endocrinologist | Reg No: DMC-000000